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Heart House
2400 N St. NW, Washington, DC 20037
1-202-375-6000 | 1-800-253-4636 | Fax: 1-202-375-6842
ACC.org

June 16, 2025

Acting Assistant Secretary Steven Posnack, MS, MHS
Centers for Medicare & Medicaid Services
Department of Health and Human Services
Attention: CMS-0042-NC
P.O. Box 8013
Baltimore, MD 21244-8013

Re: RIN 0938-AV68 Request for Information; Health Technology Ecosystem

Dear Acting Assistant Secretary Posnack,

The American College of Cardiology (ACC) is a global leader dedicated to transforming cardiovascular care and improving heart health for all. For more than 75 years, the ACC has empowered a community of over 60,000 cardiovascular professionals across more than 140 countries with cutting-edge education and advocacy, rigorous professional credentials, and trusted clinical guidance. From its world-class JACC Journals and NCDR registries to its Accreditation Services, global network of Chapters and Sections, and CardioSmart patient initiatives, the College is committed to creating a world where science, knowledge and innovation optimize patient care and outcomes. Learn more at www.ACC.org or connect on social media at @ACCinTouch.

Introduction

As noted in the background of this request for information, the passage of the 21st Century Act and subsequent rulemaking by the Centers for Medicare & Medicaid Services (CMS), the Assistant Secretary for Technology Policy/Office of the National Coordinator for Health Information Technology (ASTP/ONC), and the National Institute of Standards and Technology (NIST) have implemented policies that seek to enhance the amount of health data available through digital channels and give patients secure, electronic access to their personal health information. The College has largely supported efforts to put into place the building blocks of a digital ecosystem in which patients can view, manage, utilize, and share their health information through digital applications (apps) and other modern solutions. The ACC agrees that this digital ecosystem could ultimately expand access to personalized health guidance for patients to improve prevention and chronic disease management.

The cardiovascular space is an area of medicine rich with innovation and multiple instances where the development and adoption of digital health management and care navigation applications such as wearable and home-based medical devices, mobile health applications, and telehealth services all serve to improve patient care. However, for these technologies to reach their full potential, agencies

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such as ASTP/ONC must continue to foster the development of a fully interoperable, standardized healthcare ecosystem.

Since the publication of the ONC Cures Act final rule, CMS Interoperability and Patient Access final rule, and the *Health Data, Technology, and Interoperability* (HTI-1) and HTI-2 final rules, the College has been pleased as federal agencies have continued to build this ecosystem predicated on standardized electronic information exchange. As the new administration seeks to continue this progress, the ACC appreciates the CMS and ASTP/ONC solicitation of public comments to drive large-scale adoption of health management and care navigation applications, reduce barriers to data access and exchange, realize the potential of recent innovations in healthcare that promote better health outcomes, and accelerate progress towards a patient-centric learning health system.

The College is pleased to submit comments in response to the request for information, focusing on a few areas that the ACC believes can help achieve the common goals of improved interoperability, reduced regulatory burdens, and better health outcomes using data and personalized medicine.

Data Exchange

Application Programming Interfaces (APIs)

In October 2024, ASTP/ONC issued the *Health Data, Technology, and Interoperability: Patient Engagement, Information Sharing, and Public Health Interoperability* (HTI-2) Proposed Rule. Under this proposal, ASTP/ONC proposed, but did not finalize, to adopt a set of certification criteria to support data exchange between health care payers, providers, and patients. The ACC was supportive of these proposals as an example of the continued alignment of technical specifications and capabilities between ASTP/ONC and CMS to advance the principles of the 21st Century Cures Act.

Health Level Seven® (HL7®) Fast Healthcare Interoperability Resources® (FHIR®) application programing interfaces (APIs) help to expand the scope of information available to patients, providers, and payers via standardized data exchange. The College believes it is important for ASTP/ONC to work with CMS to continue to work on the development, deployment, and utilization of these APIs. As noted, these multiple proposals concerning APIs, including patient access API, provider access API, payer-to-payer API, prior authorization API, and the provider directory API have not been finalized. As ASTP/ONC considers future rulemaking, the ACC resubmits comments for consideration and to provide additional context surrounding the regulation and use of APIs by health care providers.

Patient Access API

In the proposed rule, ASTP/ONC proposed to adopt a "patient access API" certification criterion to specify requirements for Health IT Modules that can enable patients to access their health and administrative information by using a health application of their choice. **The College thanked ASTP/ONC for building on the criteria CMS requires of certain payers and aligning technical requirements for Health IT Modules to ensure patients have access to information without special effort.**

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As part of the certification criterion, ASTP/ONC proposed to require multiple registration pathways for applications to utilize this API, a functional registration pathway and a dynamic registration pathway. The ACC supported this proposed methodology to ensure the greatest number of applications can help patients access their information via APIs without sacrificing security or trust. This approach also ensures an easier, more consistent approach for patient access. To ensure security, ASTP/ONC proposes requiring multi-factor authentication for patient-facing applications, an approach the ACC supports to increase resiliency in the face of ever-growing cyber security threats against Health IT.

ASTP/ONC sought comments on the proposal to not include imaging links as a data element required through the Patient Access API. As ASTP/ONC noted, this element is not required under the CMS requirements currently. **While the College understands the need for continued alignment between CMS requirements on impacted payers and Health IT Modules, the ACC encouraged the development of forward-looking policies and requirements that continue to push interoperability.** ASTP/ONC's goal should always be to encourage access to as much patient information as possible, including imaging, and this can be achieved through enacting requirements that have later implementation deadlines, giving CMS and impacted payers time to develop policies to align with Health IT Module capabilities. This process would align with the process CMS and ASTP/ONC followed for the development of patient, provider, and payer APIs where one agency proposes requirements on regulated entities and the other agency follows suit.

Provider access API – client and Provider access API – server

CMS proposed certification criteria for Health IT Modules that would enable a health care provider to access information on patients' claims, including information about the patient's encounters, providers, organizations, locations, dates of service, diagnoses (conditions), procedures and observations. **The ACC agreed that the development of this certification criteria could further enable access by a health care provider to clinical information maintained by the payer from sources other than claims and thanked ASTP/ONC for working to align Health IT Module certification criteria with CMS' finalized Provider Access API requirements.** The College believes this standards-based approach will not only help increase adoption through alignment with other electronic health information sharing requirements implemented in the 21st Century Cures Act final rules, but it also has the potential to reduce the burdens associated with prior authorization by laying the groundwork for future automated processes.

The College believes this could enable access to information from sources other than claims, and one area that the ACC strongly recommended ASTP/ONC work with CMS to expand provider API capabilities includes the expansion of API requirements for prescription drug and/or covered outpatient drug information. While the ACC understood and this request was outside of the scope of the proposed rule, it was important to reiterate the importance of this information in managing patient care. Prescription drug and outpatient drug information access will help to preserve the continuity of care for new patients. Medications are an ever-growing part of disease management and patients should have access to such information.

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Considering this, the ACC recommended ASTP/ONC to work closely with CMS to align future capabilities and expand access to drug information via provider APIs.

As part of the certification criterion, ASTP/ONC proposed to require multiple registration pathways for applications to utilize the provider API, a functional registration pathway and a dynamic registration pathway. **The ACC supported this proposed methodology to ensure the greatest number of applications can help patients access their information via APIs without sacrificing security or trust.**

Payer-to-payer API

ASTP/ONC proposed to adopt a "payer-to-payer API" certification criterion to specify requirements for Health IT Modules that can be used by payers to support electronic exchange between payer systems when patients transition between payers. The development of these APIs will allow health data to follow the patient when they switch payers and enable improved coordination of care, increased patient empowerment, and reduced administrative burdens. **The ACC thanked ASTP/ONC for proposing to include this API functionality in the Health IT Module certification process and supports the continued alignment between CMS and ASTP/ONC on the development of APIs that enable information sharing.**

As part of the certification criterion, ASTP/ONC proposed to require multiple registration pathways for applications to utilize the payer-to-payer API, a functional registration pathway and a dynamic registration pathway. The ACC supported this proposed methodology to ensure the greatest number of applications can help patients access their information via APIs without sacrificing security or trust.

Prior authorization API – provider and prior authorization API – payer

ASTP/ONC proposed a prior authorization API – provider and prior authorization API – payer to establish requirements for Health IT Modules that can be used to facilitate a provider's request of coverage information and request for a prior authorization decision. These certifications would be based on the HL7 FHIR Da Vinci Burden Reduction Implementation Guides and represent a certification criterion that would help streamline prior authorization processes. **Coupled with requirements CMS has established to reduce the burdens on patients, clinicians, and their administrative staff, the College believes these processes provide much needed transparency for clinicians and their staffs to maneuver through the multitude of payer requirements. The ACC thanked ASTP/ONC for continuing to align Health IT Module requirements with CMS payer requirements.**

While supportive of the development of APIs that enable quicker, more transparent processes, **the ACC conveyed strong concern and opposition to the ever-growing prior authorization requirements for medical procedures, diagnostic tests, and medications across payers. We encouraged continued action to fully address the administrative burden of prior authorization, in part by being more judicious with deployment of prior authorization and reducing its utilization.** Patients receiving services that are nearly universally approved are unlikely

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to benefit from prior authorization. The increased use of prior authorization and the development of associated processes to manage cases is adding to the cost of care for clinicians, their practices, and institutions, decreases the amount of time clinicians spend with patients, and can reduce the quality and timeliness of care patients receive. **In our current health care environment, where decreasing costs and improving quality is a stated aim, it is important that both ASTP/ONC and CMS work with patients, payers, and clinicians to reduce the burdens associated with prior authorization, limit the necessity for purchasing expensive health IT systems and capabilities to effectively manage prior authorization caseloads, and focus on improving the quality-of-care patients receive.**

It is also important to reiterate the College's belief that ASTP/ONC work with CMS **to expand the scope of API requirements to include Medicare Advantage and the inclusion of prescription drug and/or covered outpatient drug information in any information sharing requirements.** While the ACC acknowledged these requests were beyond the scope of the proposed rule and ASTP/ONC's authority, expanding API-accessible information is crucial to ensuring essential prescription drug information used for disease management is available when needed, as well as encouraging major state and national payers to streamline prior authorization, adopt standards-based processes, and drive provider involvement.

Specifically, when it comes to prescription drug and/or covered outpatient drug information, it is the ACC's understanding that medications that are covered under a medical benefit are in scope for the HL7 FHIR Da Vinci IG for Prior Authorization Support, meaning the technical ability for the standardized exchange of this information is possible in a methodology that developers are familiar with. Due to this, **the College asked ASTP/ONC to work with CMS to continue to expand the scope of information available to patients, providers, and payers via APIs and include Medicare Advantage and prescription drug and/or covered outpatient drug information in the scope of future information sharing requirements.**

ASTP/ONC noted that nothing in the proposed certification criteria related to electronic prior authorization would alter requirements for covered entities to use adopted HIPAA transaction standards. Moreover, the FHIR specifications ASTP/ONC proposes to adopt for these certification criteria would not conflict with the use of the adopted HIPAA standard, and ASTP/ONC expects covered entities using technology certified to these criteria to ensure compliance with applicable requirements. ASTP/ONC notes the Department of Health and Human Services (HHS) has stated that HIPAA Administrative Simplification enforcement action will not be taken against HIPAA covered entities that choose not to use the X12 278 standard as part of an electronic FHIR prior authorization process.

In comments submitted to CMS for the *Advancing Interoperability and Improving Prior Authorization Processes* proposed rule, the ACC expressed concerns that the adoption of new standards may lead to implementation challenges and undue burdens and costs for providers. While this concern remains as standards advance and multiple regulatory agencies issue rules impacting the delivery of care, **the ACC thanked HHS, CMS, and ASTP/ONC for considering the impact the availability of**

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multiple standards can have on care and providing safe harbor for providers who choose to implement advanced standards such as FHIR prior authorization processes.

Provider directory API – health plan coverage

ASTP/ONC proposed to adopt a "provider directory API – health plan coverage" certification criterion which would specify technical requirements for Health IT Modules that can enable publishing of information regarding the providers that participate in a payer's network. These APIs can enable third parties to develop applications through which consumers and providers can query the participants in a payer's network that may provide services that address their healthcare needs. **The ACC thanked ASTP/ONC for proposing the development of these services to improve transparency and information availability for patients and providers.**

Clinical Quality Data Responsibilities

Digital Quality Measurement

In the CY 2026 Hospital Inpatient Prospective Payment Systems (IPPS) proposed rule, HHS issued a request for information on *Toward Digital Quality Measurement in CMS Quality Programs*. In this request, HHS seeks input from stakeholders on the future the use of FHIR in electronic clinical quality measure (eCQM) reporting. Should HHS move forward to require the use of FHIR-based measurement, certified electronic health records (EHRs) and the USCDI data classes and elements will be essential components of an effective reporting strategy. **This strategy should consist of a unified approach by all HHS agencies, from CMS and ASTP/ONC to FDA and CDC to align on specific standards, provide technical assistance and resources to encourage the adoption of these standards, and provide sufficient time to help with the transition.**

In comments submitted to HHS, the College indicated that the current reality is measures utilized by the entire cardiovascular team exist in multiple standards, some of which are commonly used and others of which are proprietary standards developed in close consultation with hospitals and other reporting sites. It will take extensive time and resources to make these measures FHIR-based. Currently, the College lacks the resources to fully crosswalk measures to FHIR-based standards and a lack of definitive direction from agencies prevents the ACC from investing necessary resources in this transition. **As agencies examine regulatory priorities, HHS and other agencies should provide medical societies, clinical data registries, and measure stewards with specific direction and sufficient warning before implementing major changes.**

The ACC also calls on CMS and ASTP/ONC to explore ways to reduce the complexity of the submission process, including offering centralized platforms or leveraging specialty society registries that already collect high-quality data. In many cases, registries and specialty societies can offer more accurate, clinically nuanced data that reflects the complexity of care. Many data elements used in current eCQMs are either not routinely captured in structured EHR fields or are captured inconsistently across systems. Specific clinical findings, procedure indications, and shared decision-making elements are often documented in free text or within registries, rather than in discrete EHR fields that align with QI-Core profiles. Registries, however, are nimble and adaptable and can quickly add fields to capture additional data. **The College requests agencies**

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permit the use of validated registry or third-party vendor data where appropriate, especially when it enhances the accuracy and utility of performance measurement.

Additionally, attribution logic can be particularly challenging to represent accurately in FHIR, especially for measures that span multiple providers or care settings. Without standardization of these data inputs across EHR vendors and sufficient time for mapping, validation, and workflow integration, the transition could place a significant burden on clinicians and health IT teams. **As HHS considers the shift towards FHIR-based measurement, we urge close collaboration with specialty societies and registries to identify which measures are most feasible to convert and to ensure that any transition maintains clinical accuracy and usability.** This approach is essential to ensure the prevention and mitigation of sunk costs by clinical data registries and reporting sites such as providers and hospitals.

Real-World Evidence, Clinical Trials, and Registries

The development of standardized digital quality measurement also necessitates the collection of anonymized real-world data from sources outside of the clinical workplace, including at-home devices. While many of these sits outside of the scope of ASTP/ONC regulatory jurisdiction, the development of an ecosystem that encourages the development of registries and standardized data collection processes can help to inform medical decision-making, clinical guidelines, and regulatory decisions.

This includes the development of standardized online consent processes and EHR flags for clinical trial participation to enhance transparency and coordination. Digital tools interoperable with certified EHR systems can help to facilitate remote trial participation through real-time monitoring, symptom tracking, and multilingual engagement. These strategies reduce participant burden, improve retention, especially among older adults and underserved populations, and can increase data quality. **The ACC encourages ASTP/ONC to closely collaborate with other agencies including CMS and FDA to foster the development of policies that facilitate improved standardized data exchange, clinical data registries, and sources of real-world data.**

Information Blocking

As previously stated, since the passage of the 21st Century Cures Act in 2016, there have been multiple major rules finalized, working to implement provisions of the Act and subsequently building on the promise of the legislation. These rules represent massive changes to the healthcare industry, impacting everything from contracts that govern business to shaping the software providers use every single day in practice. These changes have drastically impacted provider workflows, upended business models, and changed patient and provider expectations. While the College has largely supported this march towards a progressively more interoperable and standardized health technology ecosystem, the ACC asks CMS, ASTP/ONC, the Office of the Inspector General (OIG), and other agencies to focus on the implementation and enforcement of current regulations before moving towards additional change.

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One area specifically the College would like to see ASTP/ONC and OIG focus on is the enforcement of information blocking prohibitions. Information blocking is any practice that is likely to interfere with the access, exchange, or use of electronic health information. These regulations are crucial to improving the exchange of health information between providers and patients, enabling better health care outcomes. According to ASTP/ONC information blocking website, ASTP/ONC has received a total of 1,307 information blocking portal submissions, 1,227 of which were found to be possible claims of information blocking.

The College continues to hear of information blocking reports from our members and the weaponization of EHRs and contracts as competitive weapon, harming patients. One member stated they are an independent cardiology practice that treats patients from two separate health systems, both of which utilize the same EHR system. Despite this system being certified, one system refuses to allow non-employed or aligned practices to access patient information while the other refuses to utilize tools that allow information exchange between non-aligned sites of care. Recently, the Department of Justice (DOJ) and Federal Trade Commission (FTC) requested feedback from stakeholders on non-competitive practices and regulations that can work to improve competition and innovation. **As this becomes a priority for this administration, the ACC strongly encourages HHS leadership to require OIG to enforce ASTP/ONC investigated information blocking claims.**

Transition to Value-based Care

The request for information states the "transition to value-based care (VBC) represents a cornerstone of CMS' strategy to incentivize improvements in health outcomes rather than increases in service volume. The role of technology is critical to this transformation. While significant progress has been made in health IT adoption, opportunities remain to better align technology requirements with the needs of providers participating in Alternative Payment Models (APMs) and other value-based care programs." The College agrees with this sentiment and is encouraged to see CMS and ASTP/ONC seeking to understand potential obstacles that exist, and opportunities presented to utilize technology to help improve value-based care.

Technology offers the potential to assist in the health care system's transition to value-based care. As differing technology tools have been developed and more are forthcoming, provider's choices for patient monitoring and care tools are many. Providers can be trusted to utilize technology that improves efficiency and quality of care that fits their sub-specialty and patient needs dependent on diagnosis, condition acuity and comorbidities. Cost of these technologies can often be an issue, particularly for rural providers and those in independent practice. The College encourages financial support for these providers to ensure patients receiving their care also benefit from technological advancement.

The College see real potential for technology to answer an acute need in value-based care programs by enabling the delivery of timely and actionable data to clinician participants. As value-based programs develop, clinician adaptability and success will need to be supported by data delivery that enables programmatic changes that inform providers as they work to optimize quality and

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cost. Additionally, technology could utilize this data to provide suggestions of step-by-step plans for improvements. The College has a deep interest in supporting coordination between primary and specialty care in value-based arrangements. Technology that optimizes communication, care coordination and cost reporting between clinicians of varying specialties is also important. The success and ongoing participation of providers who take financial risks in value-based care will need to be supported by timely reporting technologies.

Conclusion

The College thanks the ASTP/ONC for the opportunity to provide comments on RIN 0938-AV68 Request for Information; Health Technology Ecosystem. The College looks forward to continuing to work with the Department of Justice and other federal agencies to promote competition and reduce barriers to innovation. If you have any questions or follow up, please contact Joseph Cody, Associate Director of Health IT and Digital Health Policy at jcody@acc.org or (202) 375-6251.

Sincerely,

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President
American College of Cardiology

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